

Proposed Prescribing Information
For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory only

PREGEB CAPSULES 75 MG
PREGEB CAPSULES 150 MG
(Pregabalin Capsules)

NAME AND STRENGTH OF ACTIVE SUBSTANCE(S)

Pregeb Capsules 75 mg

Each hard gelatin capsules contains:

Pregabalin 75 mg

Approved colours used in hard capsules shells

Pregeb Capsules 150 mg

Each hard gelatin capsules contains:

Pregabalin 150 mg

Approved colours used in hard capsules shells

DOSAGE FORM

Hard gelatin capsules

PRODUCT DESCRIPTION

Pregeb Capsules 75 mg

Size '4', hard gelatin capsule having orange opaque cap and white opaque body, imprinted '1360' on cap and '75' on body with black ink containing white to off-white powder.

Pregeb Capsules 150 mg

Size '1', hard gelatin capsule having white opaque cap and white opaque body, imprinted '1362' on cap and '150' on body with black ink containing white to off-white powder.

PHARMACODYNAMICS / PHARMACOKINETICS

Pharmacodynamics

Pharmacotherapeutic group: Antiepileptics, other antiepileptics

The active substance, pregabalin, is a gamma-aminobutyric acid analogue ((S)-3-(aminomethyl)-5-methylhexanoic acid).

Mechanism of action

Pregabalin binds to an auxiliary subunit ($\alpha 2$ - δ protein) of voltage-gated calcium channels in the central nervous system.

PHARMACOKINETICS

Absorption

Pregabalin is rapidly absorbed when administered in the fasted state, with peak plasma concentrations occurring within 1 hour following both single and multiple dose administration. Pregabalin oral bioavailability is reported to be $\geq 90\%$ and is independent of dose. Following repeated administration, steady state is achieved within 24 to 48 hours. The rate of pregabalin absorption is decreased when given with food resulting in a decrease in $C_{\max}$ by approximately 25-30 % and a delay in $t_{\max}$ to approximately 2.5 hours. However, administration of pregabalin with food has no clinically significant effect on the extent of pregabalin absorption.

Distribution

In preclinical studies, pregabalin has been reported to cross the blood brain barrier in mice, rats, and monkeys. Pregabalin has been reported to cross the placenta in rats and is present in the milk of lactating rats. In humans, the apparent volume of distribution of pregabalin following oral administration is approximately 0.56 l/kg. Pregabalin is not bound to plasma proteins.

Biotransformation

Pregabalin undergoes negligible metabolism in humans. Following a dose of radiolabelled pregabalin, approximately 98% of the radioactivity recovered in the urine was unchanged pregabalin. The N-methylated derivative of pregabalin, the major metabolite of pregabalin found in urine, accounted for 0.9% of the dose. In reported preclinical studies, there was no indication of racemisation of pregabalin S-enantiomer to the R-enantiomer.

Elimination

Pregabalin is eliminated from the systemic circulation primarily by renal excretion as unchanged drug.

Pregabalin mean elimination half-life is 6.3 hours. Pregabalin plasma clearance and renal clearance are directly proportional to creatinine clearance.

Dose adjustment in patients with reduced renal function or undergoing haemodialysis is necessary.

Linearity / non-linearity

Pregabalin pharmacokinetics is linear over the recommended daily dose range. Multiple dose pharmacokinetics are predictable from single-dose data. Therefore, there is no need for routine monitoring of plasma concentrations of pregabalin.

Renal impairment

Pregabalin clearance is directly proportional to creatinine clearance. In addition, pregabalin is effectively removed from plasma by haemodialysis (following a 4 hour haemodialysis treatment plasma pregabalin concentrations are reduced by approximately 50%). Because renal elimination is the major elimination pathway, dose reduction in patients with renal impairment and dose supplementation following haemodialysis is necessary.

Elderly (over 65 years of age)

Pregabalin clearance tends to decrease with increasing age. This decrease in pregabalin oral clearance is consistent with decreases in creatinine clearance associated with increasing age. Reduction of pregabalin dose may be required in patients who have age related compromised renal function.

INDICATION**Neuropathic pain**

Pregabalin is indicated for the treatment of peripheral and central neuropathic pain in adults.

Epilepsy

Pregabalin is indicated as adjunctive therapy in adults with partial seizures with or without secondary generalisation.

Generalised Anxiety Disorder

Pregabalin is indicated for the treatment of Generalised Anxiety Disorder (GAD) in adults.

Fibromyalgia

Pregabalin is indicated for the management of fibromyalgia.

RECOMMENDED DOSAGE AND MODE OF ADMINISTRATION**Posology**

The dose range is 150 to 600 mg per day given in either two or three divided doses.

Neuropathic pain

Pregabalin treatment can be started at a dose of 150 mg per day. Based on individual patient response and tolerability, the dose may be increased to 300 mg per day after an interval of 3 to 7 days, and if needed, to a maximum dose of 600 mg per day after an additional 7-day interval.

Epilepsy

Pregabalin treatment can be started with a dose of 150 mg per day. Based on individual patient response and tolerability, the dose may be increased to 300 mg per day after 1 week. The maximum dose of 600 mg per day may be achieved after an additional week.

Generalised Anxiety Disorder

The dose range is 150 to 600 mg per day given as two or three divided doses. The need for treatment should be reassessed regularly.

Pregabalin treatment can be started with a dose of 150 mg per day. Based on individual patient response and tolerability, the dose may be increased to 300 mg per day after 1 week. Following an additional week the dose may be increased to 450 mg per day. The maximum dose of 600 mg per day may be achieved after an additional week.

Fibromyalgia

The usual dose range for most patients is 300 to 450 mg per day given in two divided doses. Some patients may derive additional benefit at 600mg per day. Dosing should begin at 75 mg two times a day (150 mg/day) and may be increased to 150 mg two times a day (300 mg/day) within one week based on efficacy and tolerability. Patients who do not experience sufficient benefit with 300 mg/day may be further increased to 225mg two times a day (450 mg/day). If needed, in some patients, based on individual response and tolerability, the dose may be increased to maximum dosage of 600 mg/day after an additional week.

Discontinuation of pregabalin

In accordance with current clinical practice, if pregabalin has to be discontinued it is recommended this should be done gradually over a minimum of 1 week independent of the indication.

Patients with renal impairment

Pregabalin is eliminated from the systemic circulation primarily by renal excretion as unchanged drug. As pregabalin clearance is directly proportional to creatinine clearance, dose reduction in patients with compromised renal function must be individualized according to creatinine clearance (CL_{cr}), as indicated in Table 1 determined using the following formula:

$$CL_{cr} \text{ (ml/min)} = \left[\frac{[140 - \text{age (years)}] \times \text{weight (kg)}}{72 \times \text{serum creatinine (mg/dl)}} \right] (\times 0.85 \text{ for female patients})$$

Pregabalin is removed effectively from plasma by haemodialysis (50% of drug in 4 hours). For patients receiving haemodialysis, the pregabalin daily dose should be adjusted based on renal function. In addition to the daily dose, a supplementary dose should be given immediately following every 4-hour haemodialysis treatment.

Table 1. Pregabalin dose adjustment based on renal function

Creatinine clearance (CL _{cr}) (mL/min)	Total pregabalin daily dose*		Dose regimen
	Starting dose (mg/day)	Maximum dose (mg/day)	
≥ 60	150	600	BID or TID
≥ 30 - < 60	75	300	BID or TID
≥ 15 - < 30	25 – 50	150	Once Daily or BID
< 15	25	75	Once Daily
Supplementary dosage following haemodialysis (mg)			
	25	100	Single dose ⁺

TID = Three divided doses

BID = Two divided doses

* Total daily dose (mg/day) should be divided as indicated by dose regimen to provide mg/dose

+ Supplementary dose is a single additional dose

Patients with hepatic impairment

No dose adjustment is required for patients with hepatic impairment.

Pediatrics population

The safety and efficacy of Pregabalin in children below the age of 12 years and in adolescents (12-17 years of age) have not been reported. No data are available.

The use in children is not recommended.

Elderly (over 65 years of age) population

Elderly patients may require a dose reduction of pregabalin due to a decreased renal function.

Mode of Administration

Pregabalin may be taken with or without food.

Pregabalin is for oral use only.

CONTRAINDICATIONS

Hypersensitivity to the active substance or to any of the excipients

WARNINGS AND PRECAUTIONS

Diabetic patients

Some diabetic patients who gain weight on pregabalin treatment may need to adjust hypoglycaemic medicinal products.

Hypersensitivity reactions

Pregabalin should be discontinued immediately if symptoms of angioedema, such as facial, perioral, or upper airway swelling occur.

Dizziness, somnolence, loss of consciousness, confusion, and mental impairment

Pregabalin treatment has been associated with dizziness and somnolence, which could increase the occurrence of accidental injury (fall) in the elderly population. Therefore, patients should be advised to exercise caution until they are familiar with the potential effects of the medicinal product.

Vision-related effects

Patients should be advised they might experience visual adverse reactions including loss of vision, visual blurring or other changes of visual acuity, many of which might be transient. Discontinuation of pregabalin may result in resolution or improvement of these visual symptoms.

Renal failure

Reversible renal failure may occur with discontinuation of pregabalin.

Withdrawal of concomitant antiepileptic medicinal products

There are insufficient data for the withdrawal of concomitant antiepileptic medicinal products, once seizure control with pregabalin in the add-on situation has been reached, in order to reach monotherapy on pregabalin.

Withdrawal symptoms

After discontinuation of short-term and long-term treatment with pregabalin withdrawal symptoms such as insomnia, headache, nausea, anxiety, diarrhoea, flu syndrome, nervousness, depression, pain, convulsion, hyperhidrosis and dizziness may occur. The patient should be informed about this at the start of the treatment.

Convulsions, including status epilepticus and grand mal convulsions, may occur during pregabalin use or shortly after discontinuing pregabalin.

Concerning discontinuation of long-term treatment of pregabalin, data suggest that the incidence and severity of withdrawal symptoms may be dose-related.

Congestive heart failure

Elderly cardiovascular compromised patients are at risk to develop congestive heart failure during pregabalin treatment for a neuropathic indication. Pregabalin should be used with caution in these patients. Discontinuation of pregabalin may resolve the reaction.

Treatment of central neuropathic pain due to spinal cord injury

The incidence of adverse reactions in general, central nervous system adverse reactions and especially somnolence may increase. This may be attributed to an additive effect due to concomitant medicinal products (e.g. anti-spasticity agents) needed for this condition. This should be considered when prescribing pregabalin in this condition.

Potential for an increase in risk of suicidal thoughts or behaviors

Suicidal ideation and behaviour have been reported in patients treated with anti-epileptic agents in several indications. The mechanism of this risk is not known and the available data do not exclude the possibility of an increased risk for pregabalin.

Therefore patients should be monitored for signs of suicidal ideation and behaviours and appropriate treatment should be considered. Patients (and caregivers of patients) should be advised to seek medical advice should signs of suicidal ideation or behaviour emerge.

Reduced lower gastrointestinal tract function

Patients may experience reduced lower gastrointestinal tract function (e.g., intestinal obstruction, paralytic ileus, constipation) when pregabalin was co-administered with medications that have the potential to produce constipation, such as opioid analgesics. When pregabalin and opioids will be used in combination, measures to prevent constipation may be considered (especially in female patients and elderly).

Abuse potential

Caution should be exercised in patients with a history of substance abuse and the patient should be monitored for symptoms of pregabalin abuse.

Encephalopathy

Patients with underlying conditions that may precipitate encephalopathy are at risk to develop encephalopathy.

Effects on ability to drive and use machines

Pregabalin may cause dizziness and somnolence and therefore may influence the ability to drive or use machines. Patients are, advised not to drive, operate complex machinery or engage in other potentially hazardous activities until it is known whether this medication affects their ability to perform these activities.

INTERACTIONS WITH OTHER MEDICAMENTS

Since pregabalin is predominantly excreted unchanged in the urine, undergoes negligible metabolism in humans (< 2 % of a dose recovered in urine as metabolites), does not inhibit drug metabolism *in vitro*, and is not bound to plasma proteins, it is unlikely to produce, or be subject to, pharmacokinetic interactions.

Oral contraceptives, norethisterone and/or ethinyl oestradiol

Co-administration of pregabalin with the oral contraceptives norethisterone and/or ethinyl oestradiol does not influence the steady-state pharmacokinetics of either substance.

CNS influencing medical products

Pregabalin may potentiate the effects of ethanol and lorazepam. Pregabalin appears to be additive in the impairment of cognitive and gross motor function caused by oxycodone.

STATEMENT ON USAGE DURING PREGNANCY AND LACTATION**Women of childbearing potential / Contraception in males and females**

As the potential risk for humans is unknown, effective contraception must be used in women of child bearing potential.

Pregnancy

There are no adequate data from the use of pregabalin in pregnant women. The potential risk for humans is unknown. Pregabalin should not be used during pregnancy unless clearly necessary (if the benefit to the mother clearly outweighs the potential risk to the foetus).

Breast-feeding

It is not known if pregabalin is excreted in the breast milk of humans. Therefore, breastfeeding is not recommended during treatment with pregabalin.

Fertility

There are no clinical data on the effects of pregabalin on female fertility

ADVERSE EFFECTS / UNDESIRABLE EFFECTS

The most commonly reported adverse reactions were dizziness and somnolence. Adverse reactions were usually mild to moderate in intensity. The most common adverse reactions resulting in discontinuation from pregabalin treatment groups were dizziness and somnolence.

In the table below all adverse reactions are listed by class and frequency (very common ($\geq 1/10$); common ($\geq 1/100$ to $< 1/10$); uncommon ($\geq 1/1,000$ to $< 1/100$); rare ($\geq 1/10,000$ to $< 1/1,000$); very rare ($< 1/10,000$), not known (cannot be estimated from the available data).

Within each frequency grouping, undesirable effects are presented in order of decreasing seriousness.

The adverse reactions listed may also be associated with the underlying disease and / or concomitant medicinal products.

In the treatment of central neuropathic pain due to spinal cord injury the incidence of adverse reactions in general, CNS adverse reactions and especially somnolence was increased.

System Organ Class	Adverse drug reactions
Infections and infestations	
Common	Nasopharyngitis
Blood and lymphatic system disorders	
Uncommon	Neutropenia
Immune system disorders	
Uncommon	Hypersensitivity
Rare	Angioedema, allergic reaction
Metabolism and nutrition disorders	
Common	Appetite increased
Uncommon	Anorexia, hypoglycaemia
Psychiatric disorders	
Common	Euphoric mood, confusion, irritability, disorientation, insomnia, libido decreased
Uncommon	Hallucination, panic attack, restlessness, agitation, depression, depressed mood, elevated mood, aggression, mood swings, depersonalisation, word finding difficulty, abnormal dreams, libido increased, anorgasmia, apathy

Rare	Disinhibition
Nervous system disorders	
Very Common	Dizziness, somnolence, headache
Common	Ataxia, coordination abnormal, tremor, dysarthria, amnesia, memory impairment, disturbance in attention, paraesthesia, hypoaesthesia, sedation, balance disorder, lethargy
Uncommon	Syncope, stupor, myoclonus, loss of consciousness, psychomotor hyperactivity, dyskinesia, dizziness postural, intention tremor, nystagmus, cognitive disorder, mental impairment, speech disorder, hyporeflexia, hyperaesthesia, burning sensation, ageusia, malaise
Rare	Convulsions, parosmia, hypokinesia, dysgraphia
Eye disorders	
Common	Vision blurred, diplopia
Uncommon	Peripheral vision loss, visual disturbance, eye swelling, visual field defect, visual acuity reduced, eye pain, asthenopia, photopsia, dry eye, lacrimation increased, eye irritation
Rare	Vision loss, keratitis, oscillopsia, altered visual depth perception, mydriasis, strabismus, visual brightness
Ear and labyrinth disorders	
Common	Vertigo
Uncommon	Hyperacusis
Cardiac disorders	
Uncommon	Tachycardia, atrioventricular block first degree, sinus bradycardia, congestive heart failure
Rare	QT prolongation, sinus tachycardia, sinus arrhythmia
Vascular disorders	
Uncommon	Hypotension, hypertension, hot flushes, flushing, peripheral coldness
Respiratory, thoracic and mediastinal disorders	
Uncommon	Dyspnoea, epistaxis, cough, nasal congestion, rhinitis, snoring, nasal dryness
Rare	Pulmonary oedema, throat tightness,
Gastrointestinal disorders	

Common	Vomiting, nausea, constipation, diarrhoea, flatulence, abdominal distension, dry mouth
Uncommon	Gastrooesophageal reflux disease, salivary hypersecretion, hypoaesthesia oral
Rare	Ascites, pancreatitis, swollen tongue, dysphagia
Skin and subcutaneous tissue disorders	
Uncommon	Rash papular, urticaria, hyperhidrosis, pruritus
Rare	Stevens Johnson syndrome, cold sweat
Musculoskeletal and connective tissue disorders	
Common	Muscle cramp, arthralgia, back pain, pain in limb, cervical spasm
Uncommon	Joint swelling, myalgia, muscle twitching, neck pain, muscle stiffness
Rare	Rhabdomyolysis
Renal and urinary disorders	
Uncommon	Urinary incontinence, dysuria
Rare	Renal failure, oliguria, urinary retention
Reproductive system and breast disorders	
Common	Erectile dysfunction
Uncommon	Sexual dysfunction, ejaculation delayed, dysmenorrhoea, breast pain
Rare	Amenorrhoea, breast discharge, breast enlargement, gynaecomastia
General disorders and administration site conditions	
Common	Oedema peripheral, oedema, gait abnormal, fall, feeling drunk, feeling abnormal, fatigue
Uncommon	Generalised oedema, face oedema, chest tightness, pain, pyrexia, thirst, chills, asthenia
Investigations	
Common	Weight increased
Uncommon	Blood creatine phosphokinase increased, alanine aminotransferase increased, aspartate aminotransferase increased, blood glucose increased, platelet count decreased, blood creatinine increased, blood potassium decreased, weight decreased

Rare	White blood cell count decreased
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After discontinuation of short-term and long-term treatment with pregabalin withdrawal symptoms have been reported in some patients. The following reactions have been mentioned: insomnia, headache, nausea, anxiety, diarrhoea, flu syndrome, convulsions, nervousness, depression, pain, hyperhidrosis and dizziness, suggestive of physical dependence. The patient should be informed about this at the start of the treatment.

Concerning discontinuation of long-term treatment of pregabalin, data suggest that the incidence and severity of withdrawal symptoms may be dose-related.

OVERDOSE AND TREATMENT

The most common adverse reactions observed with pregabalin include somnolence, confusional state, agitation, and restlessness and rarely coma.

Management of Overdose

Treatment of pregabalin overdose should include general supportive measures and may include haemodialysis if necessary.

STORAGE CONDITIONS:

Store below 30°C, Protect from light and moisture.

Keep out of reach of children.

PACKAGING AVAILABLE:

Pregeb Capsules are packed in Alu-Alu blister of 10 capsules. Such blisters containing 10 capsules are packed into boxes of 10's, 30's, 60's and 100's. Not all presentations may be available locally.

DATE OF REVISION OF PACKAGE INSERT:

16/07/2015

Manufactured by:



TORRENT PHARMACEUTICALS LTD.

Indrad-382 721, Dist. Mehsana, INDIA.